

[Rights, The; Deliberate Indifference; Habeas Corpus Writs; Legitimate Penological Interests; Prison Litigation Reform Act; \*Ruffin v. Commonwealth\*; Section 1983 Lawsuits](#)

### Further Reading

Antiterrorism and Effective Death Penalty Act, 18 U.S.C. § 101–108 (1996).

*Bounds v. Smith*, 430 U.S. 817 (1977).

*Brown v. Plata*, 563 U.S. 493 (2011).

*Cooper v. Pate*, 378 U.S. 546 (1964).

*Johnson v. Avery*, 393 U.S. 483 (1969).

*Jones v. Cunningham*, 371 U.S. 236 (1963).

*Lewis v. Casey*, 516 U.S. 804 (1996).

*McQuiggin v. Perkins*, 133 S. Ct. 1924 (2013).

*Pervear v. Commonwealth*, 72 U.S. 475 (1867).

Prison Litigation Reform Act, H.R. 3019, 104th Cong. (1996).

*Ruffin v. Commonwealth*, 62 Va. 790 (1871).

## Juvenile Detention Centers

Juvenile detention centers are responsible for holding juvenile offenders whose conduct is subject to court jurisdiction. Detention centers are equivalent to adult jails in which the main purpose is for temporarily holding youth who are likely to commit another crime prior to adjudication or who are likely to fail to appear at an upcoming court date. As with jails, detention centers also hold adjudicated juveniles who have been given short-term sentences (typically less than six months). Detention centers may be administered by city, county, or state governments. However, in most cases, the county, in conjunction with the juvenile court and the probation department, is responsible for the daily administration.

Theoretically, juvenile detention centers are supposed to be reserved for serious juvenile offenders who pose a serious threat to either themselves or society; however, research has found that most detained youth do not fit this criteria. In fact, in 2013, only 26 percent of detained youth had committed a person offense. The other nearly two-thirds of youth being held were charged with nonviolent offenses including property, drug, and public order offenses as well as technical probation violations and status offenses (noncriminal acts considered law violations solely because of a juvenile's status as a minor).

Detention rates not only vary by offense but by multiple demographic characteristics as well, including gender, age, and race. Male offenders tend to be more likely to be detained than female offenders. Additionally, older youth (16 or older) are more likely to be detained than younger youth (15 or younger). Race represents the greatest variation among delinquency cases. Black juvenile offenders are detained at a disproportionately higher rate than white juvenile offenders for all types of offenses. To illustrate, in 2004, black juvenile offenders accounted for 29 percent of delinquency cases handled by the court; however, the proportion of detained youth who were black was 37 percent. This trend has been consistent since at least 1985. These findings are not unique to detention decisions as it has been found that blacks are overrepresented in all stages of the juvenile justice system.

The idea of detaining youth separately from adults came about during the 19th century when society's perception of children and their criminal responsibility began to shift. Specifically, in 1822, the Society for the Prevention of Pauperism, a group that was influential in the reformation of the treatment of juveniles, issued a report recommending the establishment of a separate penitentiary for juvenile offenders. The New York House of Refuge was created three years later, representing the first institution to deal solely with the care and confinement of juvenile offenders. Following its creation and a good reception by the public, numerous other cities followed suit, building their own houses of refuge.

Municipal and state governments began to play an increasing role in the creation and administration of juvenile institutions during the mid-19th century. As part of this shift, there was an increased emphasis on formal education, as it was believed that educating and properly training youthful offenders could help to offset the numerous factors contributing to their delinquent behavior, such as poverty and a poor family environment. To

illustrate this new emphasis, houses of refuge were renamed reform schools. With the exception of the South, by 1890, almost every state had developed some type of reform school for boys, and many even had separate institutions for girls.

Although the initial intent of reform schools was to rehabilitate youth and prepare them to be productive members of society, this idea took a blow during the mid-1970s when Robert Martinson published his infamous study “What Works?—Questions and Answers about Prison Reform,” which concluded that nothing works when it came to rehabilitating offenders. Following this report, detention centers began to more closely resemble adult jails, emphasizing punishment as opposed to rehabilitation. The erosion of rehabilitation continued throughout the 1980s and 1990s as juvenile crime increased and the public feared the rise of “juvenile superpredators.”

Also during this time, detention rates more than doubled. From 1985 to 1995, there was a 68 percent increase in the one-day detention rate. Detention rates continued to increase until they peaked in 2002. Since that time, they have been steadily decreasing. As of 2013, detention rates were at their lowest level since at least 1985. In conjunction with the growing emphasis to reduce the number of detained youth, there has also been a renewed interest in rehabilitation, which has resulted in the implementation of various types of programming.

The majority of programming available within detention facilities is focused on education and mental health. Educational programming is required in all facilities due to compulsory education laws. The first of these laws was enacted by Rhode Island in 1840, and by 1918 every state had enacted similar statutes. These laws require that state legislatures provide free education for all juveniles, including juvenile delinquents, residing within the state. Although states are required to provide educational services in detention centers, in many cases, little attention is paid to the quality of education provided. Institutions vary on the educational services available and the quality of such services. In some cases, facilities lack designated classrooms, instructional technology, libraries, and even books. Further, teachers within detention facilities are often poorly trained and incapable of dealing with the needs of detained youth.



Youthful offenders watch television at a juvenile detention facility in Toledo, Ohio. As is the case with adult incarceration, the vast majority of inmates in juvenile detention facilities are male. (Melanie Stetson Freeman/The Christian Science Monitor via Getty Images)

In order to try to combat these problems, the federal No Child Left Behind Act of 2001 required states receiving federal funding for education to monitor and improve correctional education services. However, states violating the requirements of the act have failed to face major consequences, thus giving states little incentive to make changes. Although the quality of educational programming available in juvenile detention centers is still lacking, hopefully, with the renewed focus on rehabilitation, efforts will be made to improve the quality of educational services provided to detained youth.

Many detained juveniles have been diagnosed with a mental health illness. Research has found that approximately 65 to 70 percent of youth in the juvenile justice system have a diagnosable mental illness. Additionally, more than 60 percent of youth with a mental health disorder also have a substance use disorder. Recognizing that in order to rehabilitate youth their illnesses must be addressed, detention centers have implemented a variety of different programs focused on mental health and substance use issues. Minimal data exist on available mental health programming in detention centers; however, according to a study conducted during the 1990s by the Office of Juvenile Justice and Delinquency Prevention (OJJDP) on conditions of confinement in juvenile justice facilities, mental health professionals were available in 87 percent of facilities with detained youth. The majority of services available during this time focused on individual and family counseling, substance use and abuse, and suicide prevention.

A relatively new emphasis has been on implementing trauma-informed care in juvenile detention centers. Trauma-informed care is a treatment framework that involves understanding the effects of trauma and how to appropriately respond to it. Thus, for a detention center, trauma-informed care would involve implementing strategies and policies aimed at minimizing the effects of institutional trauma as well as addressing prior traumatic experiences. Strategies that have been implemented to reduce trauma include front door screening and orientation, staff training, behavioral interventions, and reforming institutional values. Trauma-informed care has achieved some success, but the practice is not yet universal and further research needs to be conducted in order to determine the most effective practices.

Despite the renewed emphasis on programming and rehabilitation, research has found that detention has a negative impact on multiple areas of a juvenile's life, including the likelihood of reoffending, mental health, and education. One of the main arguments for detaining youth is that it will keep them from engaging in additional crimes. It has been found, however, that detaining youth may actually increase the likelihood of reoffending. In fact, one study of incarcerated youth in Arkansas found that prior commitment was the most significant predictor of recidivism. Researchers have cited social learning theory to explain this phenomenon. It is argued that congregating delinquent youth together allows them the opportunity to learn antisocial attitudes and behaviors from their peers.

Detention can also negatively impact a youth's mental health. As noted above, the majority of detained youth enter detention with some mental health issue. Often, the conditions of confinement, such as overcrowding, enhance one's condition. Further, it has been found that some youth develop a mental illness while being detained. One study found that one-third of incarcerated youth diagnosed with depression developed the condition during their detention stay.

Finally, detention has been found to impact detained youths' education. Studies have found that many detained youth fail to return to school upon release. A study conducted by the Department of Education found that 43 percent of youth receiving remedial education services in detention failed to return to school after release. An additional 16 percent enrolled in school but ultimately dropped out. Due to the potential negative effects of detention, its use should be reserved for the most serious offenders.

Detaining juveniles in facilities separate from adults has been a common practice in the United States for almost two centuries, with its main purpose fluctuating between treatment and punishment. In recent decades, there has been a shift toward reducing the number of youth detained as well as increasing the available programming in detention facilities. As society continues to change, so too will the role and purpose of juvenile detention centers.

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See also: [Juveniles in Corrections](#), [Landmark Cases Involving](#); [Sight and Sound of Juvenile Offenders](#); [State-Raised Youths](#)

### Further Reading

- Burrell, S. (2013). *Trauma and the environment of care in juvenile institutions*. Los Angeles: National Center for Child Traumatic Stress.
- Desai, R. A., Goulet, J. L., Robbins, J., Chapman, J. F., Migdole, S. J., & Hoge, M. A. (2006). Mental health care in juvenile detention facilities: A review. *Journal of the American Academy of Psychiatry and the Law*, 34(2), 204–214.
- Holman, B., & Ziedenberg, J. (2006). *The dangers of detention: The impact of incarcerating youth in detention and other secure facilities*. Washington, DC: Justice Policy Institute.
- Pickett, R. S. (1969). *House of refuge: Origins of juvenile reform in New York State*. Syracuse, NY: Syracuse University Press.

Platt, A. M. (1969). *The child savers: The invention of delinquency*. Chicago: University of Chicago Press.

Twomey, K. (2008). The right to education in juvenile detention under state constitutions. *Virginia Law Review*, 94(3), 765–811.

## Juveniles in Corrections, Landmark Cases Involving

During the 1960s and 1970s, the Supreme Court decided a number of juvenile court cases. However, the majority of these cases dealt with due process proceedings in the juvenile court. The court heard relatively few cases dealing with juvenile corrections; of these cases, they have focused largely on the sentencing of juveniles in adult courts. While the Supreme Court has heard few cases involving juvenile corrections, federal circuit courts have made many important rulings regarding juvenile corrections, specifically juveniles' right to treatment and conditions of confinement.

One issue that the Supreme Court has dealt with three separate times is the constitutionality of the juvenile death penalty. The Supreme Court first ruled on